

D345

Psychopharmacology for Advanced Psychiatric Mental Health Practice

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A patient starts a once-daily medication with a half-life of about 24 hours. She is a retired librarian who keeps a detailed medication diary. About how long until blood levels reach steady state?

- A. About 1 day
- B. About 4-5 days
- C. About 6 weeks, when full clinical benefit appears
- D. Never; levels keep rising with every dose

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

A drug level is 200 ng/mL when it is stopped. Its half-life is 36 hours. What is the expected level 108 hours later?

- A. 50 ng/mL
- B. 25 ng/mL
- C. 66.7 ng/mL
- D. 0 ng/mL

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A patient stable on a drug cleared mainly by CYP2D6 is started on fluoxetine, a strong CYP2D6 inhibitor. He works as a long-haul truck driver and drinks one cup of coffee a day. What should the PMHNP anticipate?

- A. Caffeine will offset the interaction.
- B. Lower levels and loss of effect.
- C. No change, because fluoxetine is an SSRI.
- D. Higher levels of the first drug and more side effects; monitor and consider a dose reduction.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

A patient on a CYP3A4-metabolized medication is started on carbamazepine, a CYP3A4 inducer. What happens to the first medication's level?

- A.** It is unaffected because inducers only affect CYP2D6.
- B.** It rises only on the first day.
- C.** It falls, which may reduce effectiveness.
- D.** It rises toward toxicity.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

Two hours after tramadol was added to her sertraline, a woman who teaches high school chemistry becomes agitated with a temperature of 38.8 °C, sweating, hyperreflexia, and inducible clonus in both ankles. What is the most likely cause?

- A.** Serotonin syndrome
- B.** Normal SSRI start-up effects
- C.** Anticholinergic toxicity
- D.** Neuroleptic malignant syndrome

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

Five days after starting haloperidol, a patient develops high fever, lead-pipe rigidity, confusion, and a very high CK. Which syndrome is most likely?

- A.** Akathisia
- B.** Neuroleptic malignant syndrome
- C.** Tardive dyskinesia
- D.** Serotonin syndrome

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A patient stable on lithium for bipolar disorder sprains her ankle and plans to take ibuprofen daily. She lives alone with two cats and gardens most weekends. What is the key concern?

- A. Ibuprofen will lower lithium levels and trigger mania.
- B. Over-the-counter drugs never interact with lithium.
- C. Gardening in hot weather is the main issue, not the ibuprofen.
- D. Ibuprofen can raise lithium levels toward toxicity; consider acetaminophen or monitor levels closely.

Target word _____	The one fact that decides _____	Answer ____	[] sure [] unsure [] guess
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8.

A PMHNP explains aripiprazole to a patient who is a college basketball player and previously took haloperidol. How is aripiprazole's action at the D2 receptor best described?

- A. Partial agonist: it stimulates D2 receptors less than dopamine and blocks excess dopamine.
- B. Full antagonist, the same as haloperidol.
- C. Full agonist that raises dopamine activity.
- D. Serotonin reuptake inhibitor.

Target word _____	The one fact that decides _____	Answer ____	[] sure [] unsure [] guess
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9.

An 82-year-old widower who lives in an assisted-living apartment and plays bridge twice a week has insomnia. His daughter asks about diphenhydramine or a benzodiazepine. Which approach is most appropriate?

- A. A benzodiazepine, since he is socially active and low risk.
- B. Diphenhydramine at the usual adult dose.
- C. Start with sleep hygiene or CBT for insomnia, and if medication is needed, choose a safer option at a low dose and titrate slowly.
- D. Never prescribe any psychotropic to adults over 80.

Target word _____	The one fact that decides _____	Answer ____	[] sure [] unsure [] guess
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10.

A 21-year-old who works at a bike shop started sertraline 10 days ago and says she feels no better. She has read many online reviews of the drug. What is the best response?

- A.** Double the dose today for faster results.
- B.** Rely on online reviews to decide whether to continue.
- C.** Explain that full effect often takes 4–8 weeks, continue if tolerated, and monitor closely for worsening mood or suicidal thoughts.
- D.** Switch to a different antidepressant now.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A man with opioid use disorder who works in construction wants to start naltrexone. He last used heroin 2 days ago. He has two children and a supportive partner. What is the key concern?

- A.** Starting now could cause precipitated withdrawal; he should be opioid-free about 7–10 days first.
- B.** Naltrexone can start immediately because it is not an opioid.
- C.** His family support removes the withdrawal risk.
- D.** He should start methadone the same day in the clinic office.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12. [TRANSFER — reversed stem / different variable]

A patient plans to begin buprenorphine for opioid use disorder. When is the standard time to take the first dose?

- A.** Right after his last opioid use, before withdrawal begins.
- B.** Only after 7–10 fully opioid-free days.
- C.** Any time, since buprenorphine is an opioid and cannot cause withdrawal.
- D.** Once he shows objective signs of mild-to-moderate withdrawal.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer B Concept 1

A patient starts a once-daily medication with a half-life of about 24 hours. ~~She is a retired librarian who keeps a detailed medication diary.~~ About how long until blood levels reach steady state?

Target word: time to steady state **Decides:** 4-5 half-lives × 24 hours.

A	Dropped element	One half-life, not the 4-5 needed.
B	Correct	4-5 half-lives × 24 hours = 96-120 hours.
C	Confusion set	Clinical response time is not pharmacokinetic steady state.
D	Absolute language	Intake and elimination balance after 4-5 half-lives.

2. Answer B Concept 1 · transfer

A drug level is 200 ng/mL when it is stopped. Its half-life is 36 hours. What is the expected level 108 hours later?

Target word: remaining level after elimination **Decides:** $108 / 36 = 3$ half-lives.

A	Dropped element	Counts only two half-lives.
B	Correct	$200 \rightarrow 100 \rightarrow 50 \rightarrow 25$.
C	Inverted operation	Divides by 3 instead of halving three times.
D	Absolute language	Three half-lives leaves 12.5%, not zero.

3. Answer D Concept 2

A patient stable on a drug cleared mainly by CYP2D6 is started on fluoxetine, a strong CYP2D6 inhibitor. ~~He works as a long-haul truck driver and drinks one cup of coffee a day.~~ What should the PMHNP anticipate?

Target word: direction of interaction **Decides:** Inhibitor slows metabolism.

A	Camouflage	One cup of coffee does not decide this 2D6 interaction.
B	Inverted operation	That is what an inducer does.
C	Dropped element	Ignores fluoxetine's CYP2D6 inhibition.
D	Correct	Inhibiting 2D6 slows clearance and raises levels.

4. Answer C Concept 2 · transfer

A patient on a CYP3A4-metabolized medication is started on carbamazepine, a CYP3A4 inducer. What happens to the first medication's level?

Target word: inducer effect **Decides:** Inducer speeds metabolism.

A	Confusion set	Carbamazepine induces 3A4 among others.
B	Adjacent category	Induction develops over days to weeks and lowers levels.
C	Correct	Induction speeds clearance.
D	Inverted operation	That is the effect of an inhibitor.

5. Answer A Concept 3

Two hours after tramadol was added to her sertraline, a woman ~~who teaches high school chemistry~~ becomes agitated with a temperature of 38.8 °C, sweating, hyperreflexia, and inducible clonus in both ankles. What is the most likely cause?

Target word: which syndrome **Decides:** Rapid onset, serotonergic combination, clonus.

A	Correct	Serotonergic drugs, rapid onset, clonus and hyperreflexia.
B	Dropped element	Ignores fever and clonus, which are warning signs.
C	Adjacent category	Causes dry, hot skin rather than sweating and clonus.
D	Confusion set	NMS follows dopamine blockers, develops over days, with lead-pipe rigidity.

6. Answer B Concept 3 · transfer

Five days after starting haloperidol, a patient develops high fever, lead-pipe rigidity, confusion, and a very high CK. Which syndrome is most likely?

Target word: reverse: identify NMS **Decides:** Dopamine blocker, slow onset, rigidity, high CK.

A	Inverted operation	Akathisia is restlessness, not rigidity with fever.
B	Correct	Classic NMS picture after an antipsychotic.
C	Adjacent category	TD is a late, chronic movement disorder without fever.
D	Confusion set	Serotonin syndrome shows clonus and rapid onset after serotonergic drugs.

7. Answer D Concept 4

A patient stable on lithium for bipolar disorder sprains her ankle and plans to take ibuprofen daily. ~~She lives alone with two cats and gardens most weekends.~~ What is the key concern?

Target word: lithium interaction **Decides:** NSAIDs reduce lithium clearance.

A	Inverted operation	NSAIDs raise lithium levels.
B	Absolute language	Several common OTC drugs do.
C	Camouflage	Heat can matter, but the new NSAID is the stated change.
D	Correct	NSAIDs reduce renal lithium clearance.

8. Answer A Concept 5

A PMHNP explains aripiprazole to a patient ~~who is a college basketball player~~ and previously took haloperidol. How is aripiprazole's action at the D2 receptor best described?

Target word: mechanism **Decides:** Partial agonist.

A	Correct	Partial agonism stabilizes dopamine signaling.
B	Confusion set	Haloperidol is a full D2 antagonist.
C	Inverted operation	Would worsen psychosis.
D	Adjacent category	Different mechanism and drug class.

9. Answer C Concept 6

An 82-year-old widower ~~who lives in an assisted living apartment and plays bridge twice a week~~ has insomnia. His daughter asks about diphenhydramine or a benzodiazepine. Which approach is most appropriate?

Target word: safe prescribing for older adults **Decides:** Beers: avoid anticholinergics and benzodiazepines.

A	Camouflage	Activity level does not remove fall and confusion risk.
B	Dropped element	Ignores anticholinergic risk and age-based dosing.
C	Correct	Least-harm, start-low-go-slow approach.
D	Absolute language	Careful use is sometimes appropriate.

10. Answer C Concept 7

A 21-year-old ~~who works at a bike shop~~ started sertraline 10 days ago and says she feels no better. ~~She has read many online reviews of the drug.~~ What is the best response?

Target word: timeline and safety **Decides:** Full effect takes weeks; under 25 needs close monitoring.

A	Adjacent category	Rapid escalation raises side effects without speeding response.
B	Camouflage	Reviews do not guide individual treatment.
C	Correct	Too early to judge; boxed warning applies under 25.
D	Inverted operation	Ten days is not an adequate trial.

11. Answer A Concept 8

A man with opioid use disorder who works in construction wants to start naltrexone. He last used heroin 2 days ago. ~~He has two children and a supportive partner.~~ What is the key concern?

Target word: naltrexone timing **Decides:** Needs about 7–10 opioid-free days.

A	Correct	Naltrexone is an antagonist that displaces opioids.
B	Dropped element	Ignores recent opioid use.
C	Camouflage	Support helps recovery but does not change pharmacology.
D	Adjacent category	Methadone for OUD is dispensed through an opioid treatment program.

12. Answer D Concept 8 • transfer

A patient plans to begin buprenorphine for opioid use disorder. When is the standard time to take the first dose?

Target word: buprenorphine induction timing **Decides:** Partial agonist can precipitate withdrawal if full agonist is still bound.

A	Inverted operation	This is when precipitated withdrawal is most likely.
B	Confusion set	That is the naltrexone requirement.
C	Absolute language	As a high-affinity partial agonist it can precipitate withdrawal.
D	Correct	Waiting for withdrawal lowers the risk of precipitated withdrawal.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	6	
Wrong stem number	A figure copied from the prompt that is not the answer.	0	
Adjacent category	The right idea in the wrong bucket.	6	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	8	
Absolute language	Always, never, guarantee — where judgment applies.	5	
Confusion set	Every option is real; the test is exact role-matching.	6	
Camouflage	Answers a stem fact that was never the question.	5	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Half-life, steady state, and elimination	With regular dosing, steady state is reached after about 4–5 half-lives; after stopping, each half-life halves the remaining level.	Confusing steady state with the time to full clinical effect, or subtracting a fixed amount per half-life.	A half-life and a question about when levels stabilize or how much remains.
2	CYP450 interactions	An enzyme inhibitor raises blood levels of drugs metabolized by that enzyme; an inducer lowers them.	Reversing the direction of inhibitors and inducers.	Adding a drug known to inhibit or induce a CYP enzyme.
3	Serotonin syndrome vs. NMS	Serotonin syndrome: rapid onset after serotonergic drugs, clonus and hyperreflexia, agitation; NMS: slower onset after dopamine blockers, lead-pipe rigidity, high fever, high CK.	Mixing up clonus and lead-pipe rigidity or the triggering drugs.	A new drug or dose change followed by fever and neuromuscular signs.
4	Lithium safety	Lithium has a narrow therapeutic index (maintenance about 0.6–1.2 mEq/L); dehydration, NSAIDs, thiazides, and ACE inhibitors raise levels.	Thinking over-the-counter pain relievers are harmless with lithium.	A lithium patient with a new medicine, illness, or fluid loss.
5	Agonists, antagonists, partial agonists	A partial agonist activates the receptor less than a full agonist and can block a full agonist; aripiprazole (D2) and buprenorphine (mu-opioid) are examples.	Calling a partial agonist a pure antagonist.	Mechanism of aripiprazole or buprenorphine.
6	Prescribing for older adults	Start low and go slow; avoid highly anticholinergic drugs and benzodiazepines where possible (Beers criteria) because of falls, confusion, and sensitivity.	Using adult starting doses or sedating anticholinergics for sleep.	An older adult needing a new psychotropic.
7	Antidepressant onset and warnings	Some benefit may appear in 1–2 weeks, but full antidepressant effect often takes 4–8 weeks; a boxed warning covers suicidal thoughts in people under 25, so monitor closely early on.	Switching drugs after one week or skipping early monitoring.	A patient disappointed after a short trial.

#	Concept	Rule	Trap	Tell
8	Medications for opioid use disorder	Buprenorphine is started once the patient is in mild-to-moderate withdrawal to avoid precipitated withdrawal; naltrexone requires being opioid-free for about 7-10 days; methadone is a full agonist dispensed through opioid treatment programs.	Starting buprenorphine or naltrexone right after recent opioid use.	Timing of last opioid use relative to the first dose.

Course scope (public description): Psychopharmacology for Advanced Psychiatric Mental Health Practice explains the knowledge of advanced pharmacotherapeutics and why it is important to safely and appropriately prescribe agents to manage common chronic and acute mental health problems of diverse populations. This includes differences between experimental and clinical psychopharmacology. This course covers the principles of pharmacokinetics and pharmacodynamics in administration, along with patient education. This course discusses factors of addiction and substance use, including prevalence, clinical manifestations, and treatment of various disorders. Collaborative clinical services, such as group counseling, therapeutic communities, and medication support, are explored. The foundational information in psychopharmacology in this course guides students in planning individualized mental health drug management for individuals across the life span based on setting, context, and professional ethics. The following courses are prerequisites: all MSN Core courses, Advanced Pathophysiology for the Advanced Practice Nurse, Advanced Pharmacology for the Advanced Practice Nurse, and Advanced Health Assessment for the Advanced Practice Nurse.